AO-02 Newly Born

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | October 16, 2026

Scope: Use in the immediate newborn period after field, home, or facility delivery. Effective ventilation is the priority when respirations are inadequate or heart rate is below 100/min.

Immediate newborn stabilization and resuscitation

1. Birth	Initiate the cord-management plan. Determine term gestation, breathing/crying, and muscle tone.
2. All yes	Routine care: warm, maintain temperature, skin-to-skin with parent, and ongoing evaluation.
3. Any no	Warm, dry, position airway, stimulate, and clear airway only when needed. Apply preductal pulse oximetry and consider cardiac monitoring.
4. Apnea/gasping or HR <100	Begin BVM ventilation at 40-60/min. Correct mask seal, airway position, and technique until chest movement and heart-rate response are present.
5. HR remains <60	After effective ventilation, begin two-thumb encircling chest compressions coordinated 3:1 with ventilation. Use 100% oxygen and obtain UVC/IO access when available.
6. Still HR <60	Give epinephrine 0.01 mg/kg IV/IO every 3-5 minutes. Consider hypovolemia or pneumothorax when there is no response.
7. Reassess + transport	Maintain warmth; trend right-hand/wrist/upper-arm SpO2 and heart rate; notify destination early; document APGAR at 1 and 5 minutes without interrupting care.

Key treatment and disposition

Situation	Approved action
Oxygen target	Preductal target SpO2: 2 min 65-70%; 3 min 70-75%; 4 min 75-80%; 5 min 80-85%; 10 min 85-95%.
Ventilation	Visible chest movement and rising heart rate are the primary signs of effective ventilation. Do not start compressions before ventilation is effective.
Volume	For suspected hypovolemia, normal saline 10 mL/kg IV/IO; may repeat once when there is no response to epinephrine.
Glucose	AO-02 does not carry a neonatal D10 dose. After immediate newborn stabilization, use PM-02 for the approved pediatric hypoglycemia and dextrose pathway.

Do not routinely suction the airway or perform routine endotracheal suctioning for meconium. Support ventilation when needed.

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PROVIDER ACTIONS, SAFETY, AND RELEASE CONTROL

Provider-level actions

Level	Actions
EMT	Warm, dry, stimulate, position the airway, suction only for obstruction, provide BVM at 40-60/min when indicated, begin two-thumb 3:1 compressions after effective ventilation when HR remains below 60, and prevent hypothermia.
AEMT	All EMT actions plus authorized supraglottic airway support, IV/IO access, epinephrine, fluid, and glucose treatment within Claiborne neonatal standing orders.
Paramedic	All preceding care plus advanced airway management when ventilation remains ineffective and evaluation of persistent nonresponse for hypovolemia, pneumothorax, hypoglycemia, congenital disease, infection, or maternal medication effect.

Safety checks

- Apply pulse oximetry to the right hand, wrist, or upper arm for preductal values.

- Naloxone is not recommended for neonatal respiratory depression from maternal narcotics; provide ventilatory support.

- Do not interrupt resuscitation to obtain APGAR scores.

- Premature infants lose heat rapidly; use approved warming adjuncts and anticipate specialty neonatal destination needs.

Linked pathways

Pathway	Use when
PM-02 Pediatric Diabetic / Hypoglycemia	Hypoglycemia and approved pediatric dextrose pathway after immediate stabilization
AO-01 Childbirth / Labor	Maternal care, field delivery, and postpartum coordination
AR-05 Pediatric Airway	Pediatric airway support after the immediate newborn period

Source and release control

- American Heart Association and American Academy of Pediatrics. 2025 Neonatal Resuscitation Algorithm.

- Claiborne County EMS approved newborn, medication, and pediatric-hypoglycemia content.

- This is the controlled Claiborne source PDF and matches the approved Claiborne EMS app content.

- Document focused findings, interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.